

Earache - Male (Child)

Algorithm 1155 | Child (0-17 years) | Gender at birth: Male

Definition

- Qatar-localized UAT-only adult and pediatric ear-pain pathway screening for mastoid/invasive infection, sudden hearing loss, foreign body, trauma, high-risk hosts and safeguarding; not approved for production.
- Child patient.
- Male.

Initial assessment

1. [DURATION] How long has the earache lasted?
2. [YES_NO] One ear or both?
3. [YES_NO] Any fever, discharge, or hearing changes?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant; and are there swelling behind the ear, severe dizziness, facial weakness, head injury, foreign body, diabetes, immune suppression or safeguarding concerns?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, fluid intake, urine output, pain, fever, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Ear - Foreign Body

Is an object, food, insect, battery, or other foreign material known or suspected to be inside the ear?

A suspected object in the ear requires the Ear - Foreign Body pathway rather than general earache assessment.

1. Disposition level 100

Is the patient confused, difficult to wake or rapidly deteriorating, or is there severe headache/neck stiffness, facial weakness, swelling/redness behind the ear with the ear pushed outward, or a major head injury with clear/bloody ear discharge?

Sepsis, intracranial spread, facial nerve involvement, mastoid complications and skull-base injury require emergency assessment.

1. Disposition level 70

Has the earache lasted more than 2-3 days, does the person feel generally unwell or have a high temperature, is there swelling around the ear, fluid coming from the ear, hearing loss or change, something stuck in the ear, or is this a child under 2 with earache in both ears?

NHS.UK earache guidance lists these as reasons for an urgent clinical review.

1. Disposition level 50

Does the caller keep getting earaches recurrently?

NHS.UK guidance recommends a routine primary-care review for recurring earaches.

1. Disposition level 70

After higher-risk features are excluded, is any new ear pain still present without an in-person diagnosis?

This UAT pathway requires direct ear examination before assuming a benign cause or advising medication.

Advice

Emergency ear-pain precautions

- Call Qatar 999 and do not drive. Keep the patient observed, do not insert anything or add drops to the ear, and follow dispatcher instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: reduced responsiveness or seizure; worsening swelling, headache, neck stiffness or facial weakness.

Urgent earache review

- Arrange same-day or next-day medical review for these symptoms.
- Call Back If: fever worsens; hearing loss increases.

Routine earache follow-up

- Arrange a routine primary-care review for recurring earaches.
- Call Back If: earaches become more frequent.

In-person assessment for new ear pain

- Keep the ear dry, do not insert objects, irrigate or use unverified drops, and arrange in-person assessment through the Qatar pathway approved for UAT. Medication requires age, weight, pregnancy, allergy, kidney/liver and interaction checks.
- Call Back If: pain worsens; fever, swelling, discharge, dizziness or hearing change develops.

First aid

- Call Qatar 999 and do not drive. Keep the patient observed, do not insert anything or add drops to the ear, and follow dispatcher instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Earache", <https://www.nhs.uk/conditions/earache/> (page last reviewed 27 October 2025)

Provenance

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